SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Gemcitabine 800 mg/m² · 28-day schedule

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Advanced breast cancer after two or more treatment lines · NCCP 00749 · version 3 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Control advanced cancer and symptoms, with regular review of benefit and side effects.

Schedule
Gemcitabine 800 mg/m² is given on Days 1, 8 and 15 of each 28-day cycle, for six to eight cycles unless progression or unacceptable toxicity changes the plan.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Gemcitabine
ANTIMETABOLITE CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Flu-like symptoms, fatigue, rash and swelling
- ☐ Uncommon serious lung or liver injury
- ☐ Rare haemolytic uraemic syndrome and kidney injury

Discuss the treatment setting, alternatives, exact schedule, common and serious drug-specific risks, fertility and pregnancy precautions, and the monitoring and urgent-contact plan.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00749 v3.

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